BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms	Confirm BRAF V600, blood count, kidney and liver function, lactate dehydrogenase (LDH), and baseline creatine phosphokinase (CPK) (muscle enzyme) and creatinine. Check electrocardiogram (ECG), left ventricular ejection fraction (heart pumping measurement) (LVEF) (heart pumping function), medicines and skin; record vision symptoms.	Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.	Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.	After restarting reduced-dose cobimetinib, National Cancer Control Programme (NCCP) and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest computed tomography (CT), coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00373 v3: current regimen source · EMA Cotellic product information · EMA Zelboraf product information

Tests in your plan

Lactate dehydrogenase (LDH)

WHEN

Before treatment and before each 28-day cycle.

A blood result followed alongside the clinical assessment in this regimen.

A change is interpreted with other findings; it does not establish tumour growth by itself.

Creatine kinase (CK / CPK) and creatinine

WHEN

Before treatment and monthly during treatment, or sooner for symptoms.

Checks for muscle injury and its effect on the kidneys.

Report new muscle pain, weakness or dark urine promptly.

THE WHOLE TREATMENT JOURNEY

Cobimetinib + vemurafenib — advanced melanoma: treatment and checks

Advanced melanoma

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00373	Confirm BRAF V600, blood count, kidney and liver function, LDH, and baseline CPK (muscle enzyme) and creatinine. Check ECG, LVEF (heart pumping function), medicines and skin; record vision symptoms.	Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.	Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.	After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest CT, coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00373	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00373	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Days 1–21: both medicines

Three weeks

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00373

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Days 22–28: vemurafenib continues

One week

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00373

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42